

AI Backend

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Key set

Policy Documents

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Old Policy (baseline)

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1.pdf

371.8KB

X

New Policy (updated)

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2.pdf

0.9MB

X

Loaded: 1.pdf (26,087 chars)

Loaded: 2.pdf (73,948 chars)

 Healthcare Policy Content Analyzer

AI-powered policy comparison, rule extraction & provider alert generation | Cotiviti Intern Assessment -- Likitha KB

Analysis completed in 189.6 seconds

Key Changes Detected	HIGH Risks	MEDIUM Risks	Denial-Risk Rules	Documents Compared
31	6	1	34/37	2 PDFs
	↑ action required	↑ monitor		

Comparing: 1.pdf vs 2.pdf

Policy Change Summary

Extracted Coding Rules

Provider Alert

Policy Diff Viewer

⚠ All content is generated from uploaded policy content. Only changes explicitly stated in the source documents are included. Risk levels are AI-derived — see the Compliance Risks section for methodology.

Executive Summary

This update transitions evaluation and management (E/M) examinations from an organ system-based counting system to a highly detailed, bulleted-element criteria model across general and specialty-specific exams. Strict minimum bullet counts, along with shaded and unshaded border documentation rules, now dictate examination levels and serve as direct denial triggers if unmet. The updated policy also introduces administrative flexibility, allowing providers to review and update historical history elements instead of re-recording them, and establishes explicit clinical exam exclusions for pediatric and contraindicated procedures.

Key Changes

- Clarified that performing and documenting a single component (e.g., examination) at the highest level does not automatically qualify the entire E/M encounter for the highest level.

• Added a mandatory requirement that a Chief Complaint (CC) must be indicated at all history levels.

• Allowed the Chief Complaint (CC), Review of Systems (ROS), and Past, Family, and/or Social History (PFSH) to be either listed as separate elements of history or included directly within the History of Present Illness (HPI) description.

- Defined a 'brief HPI' as consisting of one to three HPI elements, and an 'extended HPI' as consisting of at least four HPI elements or the clinical status of at least three chronic or inactive conditions.

- Introduced explicit definitions for 'problem pertinent ROS' (inquires about the system directly related to the HPI problem) and 'extended ROS' (inquires about the system directly related plus a limited number of additional systems).

- Removed subsequent nursing facility care from the list of E/M categories requiring at least one specific item from two of the three history areas to qualify for a complete PFSH.

- Explicitly named the three elements of the Medical Decision Making (MDM) table in the guidelines: number of diagnoses or management options, amount and/or complexity of data to be reviewed, and risk of complications and/or morbidity or mortality.

- Replaced the general multi-system examination requirement of 'findings about 8 or more of the 12 organ systems' with a detailed bulleted-element counting system.

- Established Problem Focused Examination criteria requiring performance and documentation of 1 to 5 bulleted elements in one or more organ systems/body areas (general) or single organ system examinations.

- Established Expanded Problem Focused Examination criteria requiring performance and documentation of at least 6 bulleted elements in one or more systems/areas (general) or single organ system examinations.

- Established Detailed General Multi-System Examination criteria requiring at least 6 organ systems/body areas with at least 2 bulleted elements each, OR at least 12 bulleted elements across 2 or more systems/areas.

- Established Detailed Single Organ System Examination (except eye/psychiatric) criteria requiring performance and documentation of at least 12 bulleted elements.

- Established Detailed Single Organ System Eye and Psychiatric Examinations criteria requiring performance and documentation of at least 9 bulleted elements.

- Established Comprehensive General Multi-System Examination criteria requiring performance of all bulleted elements across at least 9 organ systems/body areas, with documentation of at least 2 bulleted elements per system/area.

- Established Comprehensive Single Organ System Examination criteria requiring performance of all bulleted elements, documentation of every element in shaded border boxes, and at least one element in unshaded border boxes.

Show 16 more changes (31 total extracted)



- Allowed measurement of any three of seven specified constitutional vital signs to be recorded by ancillary staff to satisfy the constitutional element.

- Established specialty-specific Detailed examination requirements of at least 12 bulleted elements for Ear, Nose, and Throat (ENT), Genitourinary, Neurological, Respiratory, and Skin examinations.

- Established specialty-specific Detailed examination requirements of at least 9 bulleted elements for Eye and Psychiatric examinations.

- Established that Comprehensive specialty examinations (e.g., Neurological, Psychiatric, Respiratory) require performing all bulleted elements, documenting every shaded border box element, and documenting at least one unshaded border box element.

- Implemented specific Musculoskeletal examination rules where Comprehensive level requires all 4 joint/bone/muscle/tendon bulleted elements performed/documentated across each of 4 anatomic areas, while the lower three levels count elements separately per body area.

- Implemented specific Skin examination rules where Comprehensive level requires examination of all 4 anatomic areas, while the lower three levels count each body area separately.

- Allowed providers to review and update ROS and/or PFSH obtained during an earlier encounter instead of re-recording, provided there is documented evidence of the review.

- Established strict criteria for documenting earlier ROS/PFSH updates, requiring the provider to describe any new information (or note no change) and note the date and location of the earlier encounter.

- Added pediatric clinical exam exclusions stating that use of a mirror is not required in children during examinations of the larynx or nasopharynx.

- Excluded the determination of refractive error from the visual acuity testing component.

- Excluded the measurement of intraocular pressures in pediatric patients and patients presenting with trauma or infectious disease.

- Excluded ophthalmoscopic examinations through dilated pupils if clinically contraindicated.

- Added a new denial trigger for failing to document every shaded-border element and at least one unshaded-border element when billing a Comprehensive level exam.

- Added a new denial trigger for documenting fewer than 12 bulleted elements for a Detailed level Neurological or Respiratory exam.

- Added a new denial trigger for documenting fewer than 9 bulleted elements for a Detailed level Psychiatric exam.
- Connected payer requirements to audit and validate services explicitly to their 'contractual obligation to enrollees.'

Impact on Providers

- Align physical examination performance and clinical documentation with precise bullet-counting structures and shaded/unshaded box criteria rather than overall organ system counts.
- Ensure a Chief Complaint (CC) is explicitly documented and indicated at all history levels.
- Document the status of at least three chronic or inactive conditions to satisfy the 'extended HPI' requirement when four distinct HPI elements are not documented.
- Ensure that to bill a Comprehensive Musculoskeletal exam, all 4 joint/bone/muscle/tendon elements are performed and documented for each of 4 distinct anatomic areas.
- Ensure that to bill a Comprehensive Skin exam, all 4 anatomic areas are examined and documented.
- Train or utilize ancillary staff to record at least three of the seven constitutional vital signs to support the constitutional exam element.
- When referencing historical ROS or PFSH, explicitly document the date and location of the prior encounter, along with any updated information or a clear 'no change' statement.
- Avoid unnecessary mirror use in children for laryngeal/nasopharyngeal exams, refractive error testing during visual acuity exams, and dilated ophthalmoscopic exams where contraindicated.
- Provide specific elaboration on abnormal findings in symptomatic areas, reviews of old records, and additional histories obtained from families to prevent documentation-deficiency claims denials.

Impact on Payers

- Update claims processing, editing, and clinical audit rules to validate specific bullet counts for Detailed level examinations (12 for Neurological, Respiratory, ENT, Genitourinary, and Skin; 9 for Eye and Psychiatric).
- Configure audit systems to trigger automatic denials for Comprehensive level claims that fail to document every element in shaded border boxes and at least one element in unshaded border boxes.
- Integrate logic to accept and validate historical ROS/PFSH reviews that are updated with date, location, and a change or 'no change' notation.
- Update claims payment and audit guidelines to incorporate pediatric exclusions (mirror use on larynx/nasopharynx, intraocular pressure measurements) and clinical contraindications for dilated pupil exams.
- Align audit rationale and documentation request communications to reference the payer's 'contractual obligation to enrollees' when validating sites of service, medical necessity, or coding accuracy.

Impact Summary Table

Impact on Providers	Impact on Payers
Align physical examination performance and clinical documentation with precise bullet-	Update claims processing, editing, and clinical au
Ensure a Chief Complaint (CC) is explicitly documented and indicated at all history levels	Configure audit systems to trigger automatic deni
Document the status of at least three chronic or inactive conditions to satisfy the 'extend	Integrate logic to accept and validate historical RC
Ensure that to bill a Comprehensive Musculoskeletal exam, all 4 joint/bone/muscle/tend	Update claims payment and audit guidelines to in
Ensure that to bill a Comprehensive Skin exam, all 4 anatomic areas are examined and d	Align audit rationale and documentation request
Train or utilize ancillary staff to record at least three of the seven constitutional vital sign	—
When referencing historical ROS or PFSH, explicitly document the date and location of th	—
Avoid unnecessary mirror use in children for laryngeal/nasopharyngeal exams, refractive	—
Provide specific elaboration on abnormal findings in symptomatic areas, reviews of old	—

Compliance Risks

How are risk levels assigned?

Risk levels are AI-derived based on the following criteria applied to policy language:

Level	Criteria
 HIGH	Document explicitly states denial, rejection, or non-payment as a consequence
 MEDIUM	Document states a new requirement that could affect claim processing if missed
 LOW	Process or documentation change with no explicit penalty stated in the document

These labels are not assigned by CMS. They represent this tool's interpretation of the policy text.

 **HIGH:** Failing to document every element in each box with a shaded border and at least one element in each box with an unshaded border when billing a Comprehensive level exam will result in claim denial.

Source: DENIAL TRIGGERS: 'Failure to document every element in each box with a shaded border and at least one element in each box with an unshaded border when billing a Comprehensive level exam.'

 **HIGH:** Documenting fewer than twelve bulleted elements for a Detailed level exam in Neurological or Respiratory systems will trigger a direct denial.

Source: DENIAL TRIGGERS: 'Documenting fewer than twelve bulleted elements for a Detailed level exam in Neurological or Respiratory systems.'

 **HIGH:** Documenting fewer than nine bulleted elements for a Detailed level Psychiatric exam will trigger a direct denial.

Source: DENIAL TRIGGERS: 'Documenting fewer than nine bulleted elements for a Detailed level Psychiatric exam.'

 **HIGH:** Failing to elaborate on old records reviewed or additional history obtained from family (e.g., using simple phrases like 'Old records reviewed' or 'additional history obtained from family' without elaboration) is treated as insufficient and triggers denial.

Source: DENIAL TRIGGERS: 'A notation of "Old records reviewed" or "additional history obtained from family" without elaboration is insufficient.'

 **HIGH:** Providing a notation of 'abnormal' without elaboration for exam findings of affected or symptomatic areas is insufficient and triggers denials.

Source: DENIAL TRIGGERS: 'Providing a notation of "abnormal" without elaboration for exam findings of affected or symptomatic areas.'

 **HIGH:** Failing to individually document at least ten systems when performing a complete ROS, in the absence of a notation indicating all other systems are negative, triggers denial.

Source: DENIAL TRIGGERS: 'In the absence of a notation indicating all other systems are negative, at least ten systems must be individually documented for a complete ROS.'

 **MEDIUM:** Improperly documented reviews of historical ROS and/or PFSH that omit the date, location of the earlier records, or an update notation will fail to meet history component requirements.

Source: COMPLIANCE: 'The review and update of an earlier ROS and/or PFSH must be documented by: describing any new ROS and/or PFSH information or noting there has been no change in the information; and noting the date and location of the earlier ROS and/or PFSH.'
